

Who This Reaches

ASHA Sahayak — the people behind the numbers, and where it goes next

Every figure below is from a Government of India source
SRS 2023 · Population Projections 2011–36 · TRAI · MoHFW
sources listed on the last page

India has **142.34 crore** people and this device does not serve all of them; we have never claimed it does. What it serves is one worker, at one doorstep, answering one question — and the people on the other side of that question are **mothers and their children**. This is who they are, how many, and where they live.

2.61 cr BIRTHS A YEAR IN INDIA	11.30 cr CHILDREN UNDER FIVE ALIVE TODAY	40.70 cr WOMEN OF CHILDBEARING AGE	10.29 lakh ASHAS WHO WOULD CARRY IT
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Rural and urban, side by side

SRS 2023 × projected population

The device is given to an ASHA, and about nine in ten ASHAs work rural blocks. That is not a marketing choice — it is where the births are, and where the deaths are.

PEOPLE ON THE OTHER SIDE OF THE QUESTION	RURAL	URBAN	TOTAL	RURAL SHARE
Births in a year	1.84 cr	0.77 cr	2.61 cr	71%
Mothers and newborns together, each year	3.68 cr	1.54 cr	5.22 cr	71%
Children under five, alive today	7.90 cr	3.40 cr	11.30 cr	70%
Women aged 15–49	25.76 cr	14.94 cr	40.70 cr	63%
Population	90.82 cr	51.52 cr	142.34 cr	64%

Births from the SRS 2023 crude birth rate applied by residence – rural 20.3 and urban 14.9 per 1,000 – to the projected population for 1 March 2026. Children under five from the SRS 2023 age distribution: 8.7% of the rural population and 6.6% of the urban. Women 15–49 from the same table, female column. [1][2]

The gap is not only in numbers — it is in survival

SRS 2023

PER 1,000 LIVE BIRTHS	RURAL	URBAN	HOW MUCH WORSE, RURAL	DEATHS A YEAR, RURAL
Neonatal mortality — first 28 days	21	13	62% higher	3.87 lakh
Infant mortality — first year	28	18	56% higher	5.16 lakh
Under-five mortality	33	20	65% higher	6.08 lakh

One number decides where a device like this belongs: **73.2% of all infant deaths in India are neonatal** — they happen in the first 28 days, and most of those in the first 48 hours. That is precisely the window in which no doctor is present, the ASHA is in the home, the decision is hers, and her only reference is a printed book she cannot search while holding a baby.

Neonatal, infant and under-five mortality by residence, SRS Statistical Report 2023, Statements 46, 48 and 53. The 73.2% is the neonatal share of infant deaths in the same report. Annual deaths are those rates applied to the births above. [1]

Why it has to work with no network at all

TRAI · DoT

CONNECTIVITY, QUARTER ENDING 31 DECEMBER 2025	RURAL	URBAN
Tele-density — connections per 100 people	59.63%	148.92%
Telephone subscribers	543.70 mn	762.44 mn

An urban Indian has about **two and a half connections for every one a rural Indian has**. And that is the average, not the edge: the Government’s own 4G Saturation project was created for **24,680 villages with no coverage at all**, and of **39,799** rural and remote uncovered villages planned for 4G through Digital Bharat Nidhi, **33,044 had been covered by 2025** — leaving about **6,755 villages still waiting**. An ASHA posted in one of them cannot use a cloud assistant on any day, and an ASHA posted anywhere rural cannot rely on one on the day it matters.

This device answers with **the network switched off**. Speech recognition, translation, retrieval, the language model and the voice all run on the board in the ASHA’s hand. There is no data charge, no signal to find, and nothing about the mother or child leaves the device.

What one device costs per person it reaches

derived

At **2.61 crore births** across **10.29 lakh ASHAs**, one ASHA attends about **25 births a year**. Over a five-year service life that is roughly **127 mothers and 127 newborns** from a single device.

PER DEVICE	TODAY, DEVELOPER KIT	AT VOLUME
Hardware cost	₹46,482	₹12,000
Cost per mother–newborn pair, over five years	₹366	₹95
Cost per question asked	₹0	₹0

The last line is the one that compounds. There is no per-query cost, no subscription and no bandwidth, so asking the device a hundred questions in a day costs exactly what asking it none costs. A cloud assistant is charged for every answer it gives, forever.

Where it goes next

three tiers, honestly separated

Same device, more of the book no new hardware, no new software

Today the device carries **one of the seven ASHA modules** — Module 7, child health and nutrition. The remaining six are published by the same ministry in the same form. Adding them is a content task: it widens what she can ask about to antenatal care, delivery, postnatal care, nutrition, communicable and non-communicable disease, without touching the hardware or the pipeline. This is the single largest improvement available and it is the next thing we will do.

Same device, another language a model swap

Each unit carries one language pack. BHASHINI already publishes recognition, translation and speech models for every scheduled language, so a device for Marathi, Bengali, Tamil or Odia is the same board with different weights on it — not a new build, not a new app, and not a new procurement. A district that speaks two languages buys two packs, not two products.

Same device, another worker ANM, Anganwadi worker, MPW

The ASHA is not the only frontline worker carrying a manual she cannot search. The Auxiliary Nurse Midwife, the Anganwadi worker and the multi-purpose worker each have their own printed guidance and the same problem with it. The device does not know it is talking to an ASHA; it knows what book it is holding.

Needs new work, and in one case approval not claimed as ready

Camera-based screening. The board has a camera the current application does not use. The clearest candidate is newborn jaundice from skin colour, which an ASHA currently judges by eye in poor light. This is a medical screening claim and would need Ministry of Health approval and a proper clinical validation before it goes near a mother — we are not claiming it works today.

Beyond health. Nothing in the architecture is medical. It is an offline machine that answers questions in an Indian language from a book it has been given. The same device with a different book serves agricultural extension, entitlement and scheme eligibility, or school teaching guidance — wherever a worker in a low-connectivity district has to act on a manual she cannot search.

What we do not claim

This does not serve 142 crore Indians. Today’s scope is mothers and children, and the knowledge base is one module of seven. A question about antenatal care has no answer in the book the device carries, and it is built to say so rather than guess.

The reach figures above are the population the ASHA network is positioned to serve, not users acquired. They are what one device is worth if it is in her hand, multiplied by how many hands there are. Nothing here counts a deployment that has not happened.

The mortality figures are not a promise of lives saved. They say where the deaths are and when they happen, which is why the ASHA’s doorstep is the right place for this. Proving an effect on those numbers needs a field study, and that comes after a pilot, not before it.

Sources

all Government of India

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3. **The Indian Telecom Services Performance Indicator Report**, quarter ending 31 December 2025, Telecom Regulatory Authority of India, Press Release No. 32 of 2026. Rural and urban tele-density and subscribers.
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5. **Union Health Ministry hosts ASHAs**, Ministry of Health and Family Welfare, Press Information Bureau, January 2025. Over 10.29 lakh ASHAs in service.
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